Testosterone undecanoate 1000 mg/4 mL · treatment-room card

Ventrogluteal IM injection · observe · act. Print and laminate. Version 2.2, 18 October 2026. Full SOP: https://gamsa.au/medicines/testosterone-undecanoate

1 Screen

Identity, consent, allergies, prior reaction to oily injection [1].

Defer or refer if: chest infection or breathless; previous POME; bleeding disorder or anticoagulant [1][11].

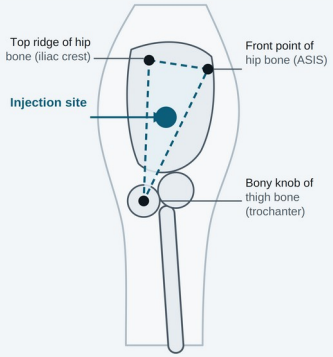
Recommended: pregnancy test before each injection if the patient could become pregnant [1][12].

Product, 1000 mg/4 mL, batch, expiry.

2 Equipment

Draw up **18G**. Inject **21G × 38 mm**; **50 mm if BMI > 30** [5][10].

Oxygen, adrenaline, monitoring, phone for 000 in the room [8].



1 Palm flat on the bony knob, fingers pointing up

2 Index finger on the front point of the hip bone

3 Spread your middle finger back along the top ridge

● Needle into the V between them, straight in at 90°

3 Landmark and inject

Side-lying, upper hip relaxed. Heel of hand on greater trochanter, index finger to ASIS, middle finger along iliac crest: inject in the **V** [4][5].

90°, deep into muscle. **Aspirate**; blood → withdraw, new needle, new site [5][10].

Full 4 mL **very slowly, about 2 minutes** [1][10].

Gentle pressure, dressing. **No massage** [5].

4 Observe

During and immediately after [1][3], and for **at least 30 minutes** under this procedure: POME typically presents within 30 minutes [12]; the PI gives no fixed time.

Record: product, batch, expiry, side, needle, time taken, clinician, outcome, next due (10–14 weeks; first interval ≥ 6 weeks) [1][3].

Book the next appointment before the patient leaves.

EMERGENCY · call 000 for severe symptoms

POME: cough, urge to cough, breathless, chest tight, sweating, dizzy, tingling, faint — within minutes

Stop. Sit up, legs out. **Oxygen.** Monitor pulse, BP, SpO_2, RR [7].

Call **000** immediately for severe breathing difficulty, collapse or signs of anaphylaxis [7][8].

Mild POME symptoms usually settle within 30 minutes to a few hours with supportive care [6][12].

ANAPHYLAXIS: rash, swelling, wheeze, stridor, hypotension, collapse

Lie flat, legs raised; sit with legs out only if breathing is the problem. Do not stand [8].

Adrenaline 1:1000 IM, outer mid-thigh: 0.01 mg/kg, max 0.5 mg (0.5 mL). Call **000**. Repeat after 5 min if no better [8].

Oxygen. Hospital observation ≥ 4 h after last dose [8].

Afterwards

Notify prescriber. Adverse event form. Report to TGA: **aems.tga.gov.au** [9].

References by number are in the SOP and training guide. Prepared by Theodore South, BPharm (Hons); a template for adaptation to local policy, not endorsed by any health service. Product Information prevails [1][11].